ROGERS
Behavioral Health

Date: 04/22/2024

RE: Kyle Rosebrook

DOB: 01/16/1986

To Whom It May Concern:

This letter is to verify that the above-named individual was receiving treatment at Rogers BehavioralHealth during the following dates.

ADMISSION

DATE: 06/22/2021
09/14/2020
07/31/2020
06/22/2020
06/07/2020

DISCHARGE

DATE: 07/29/2021
10/16/2020
09/11/2020
07/30/2020
06/17/2020

Respectfully
,

Health Information Department

34700 Valley Road, Oconomowoc, WI 53066
PHONE: 262-984-3366 FAX: 262-646-5745 WEBSITE: rogersbh.org

## Patient Information

**Patient Name:** ROSEBROOK, KYLE A
**Home Address:** 8805 W Maple St
West Allis, WI 53214
**Home Phone:** (262) 349-5504
**Employer Name:** Unknown
**Employer Phone:**

**Sex:** Male
**DOB:** 01/16/1986
**Age:** 38 Years
**Religion:** Unknown
**SSN:** XXX-XX-2138

## Guarantor Information

**Guarantor Name:** KYLE ROSEBROOK
**Patient's Reltn:** SELF
**Billing Address:** 8805 W Maple St
West Allis, WI 53214
**Billing Phone:** (262) 349-5504
**Employer Name:** Unknown
**Employer Phone:**

**Sex:** Male
**DOB:** 01/16/1986
**Age:** 38 Years
**SSN:** XXX-XX-2138

## Contact Information

***Emergency Contact***
**Contact Name:** PHYLLIS ROSEBROOK
**Patient's Reltn:** Child - EC
**Sex:** Female
**Home Phone:** 847-989-3275

***Next of Kin***
**Contact Name:**
**Patient's Reltn:**
**Sex:**
**Home Phone:**

## Primary Insurance

**Subscriber Name:** ROSEBROOK, KYLE A
**Patient's Reltn:** SELF
**Sex:** Male
**DOB:** 01/16/1986
**Age:** 38 Years
**Employer Name:** Unknown
**Employer Phone:**
**Financial Class:** Medicaid MCO

**Insurance Name:** MEDICAID HMO CCHP WI
**Claim Address:** PO BOX 56099
MADISON, WI 537059399
**Insurance Phone:** 8004828010
**Policy Number:** 6448859365
**Group Number:**
**Authorization Number:** EPS-00531657
**Authorization Phone:**
**Authorization Contact:**

## Secondary Insurance

**Subscriber Name:**
**Patient's Reltn:**
**Sex:**
**DOB:**
**Age:**
**Employer Name:**
**Employer Phone:**
**Financial Class:**

**Insurance Name:**
**Claim Address:**

**Insurance Phone:**
**Policy Number:**
**Group Number:**
**Authorization Number:**
**Authorization Phone:**
**Authorization Contact:**

## Encounter Information

**Reg Dt/Tm:** 06/22/2021 12:23
**Est Dt of Arrival:** 06/21/2021 09:00
**Inpt Adm Dt/Tm:**
**Disch Dt/Tm:** 07/29/2021 15:15
**Observation Dt/Tm:**
**VIP Indicator:**
**Admit Reason:** Depression

**Patient Type:** Intensive Outpatient
**Medical Service:** Mental Health & Add
**Location:** 4-IOP MH and Addiction R
**Room/Bed:** /
**Isolation:**
**Disease Alert:**

**Admit Type:** Urgent
**Admit Source:** Information Not Avail
**Advance Directive:** No, Info Provide
**Reg Clerk:** Kelly Tobin
**Admit Physician:**
**Attend Physician:**
**PCP:**

# ROSEBROOK, KYLE A
Male / 38 Years

**MRN:** 00153481

**FIN:** 000261514

| | | | |
|---|---|---|---|
| Patient: | **ROSEBROOK, KYLE A** | | |
| MRN: | 00153481 | Admit: | 6/22/2021 |
| FIN: | 000261514 | Disch: | 7/29/2021 |
| DOB/Age/Sex: | 1/16/1986 35 years Male | Admitting: | zzBarisic-Dujmovic MD,Tatjana |
| Location: | 4-IOP MH and Addiction Recovery Adult | Attending: | zzBarisic-Dujmovic MD,Tatjana |

## *Discharge Planning Documents*

| | |
|---|---|
| Document Type: | Discharge Summary |
| Service Date/Time: | 7/29/2021 13:17 CDT |
| Result Status: | Auth (Verified) |
| Perform Information: | Barisic-Dujmovic MD,Tatjana (7/29/2021 13:17 CDT) |
| Sign Information: | Barisic-Dujmovic MD,Tatjana (8/1/2021 14:10 CDT) |

**Chief Complaint**
See my initial psychiatric evaluation

**Admission Information**
See my initial psychiatric evaluation

**Course of Treatment**
Patient is single 35-year-old male with past psychiatric diagnosis of MDD, generalized anxiety disorder, sedative-hypnotic use disorder who has been referred to our dual PHP programming. Patient is well-known to our Rogers behavioral health system as he was in residential programming in June of last year followed by PHP and then IOP which he successfully completed. Of note there have been some relapses throughout his outpatient programming last year but he eventually discharged sometimes in October of last year. Patient was supposed to schedule aftercare but it does not seem that he pursued seeing either therapist or psychiatrist. Patient reports quickly relapsing on his prescribed Xanax. As a matter fact he admits that he has been continuing to seek Xanax prescription from his primary care last year even while he was in the treatment despite the fact that he has not been taking it and his urine drug screens were negative for benzodiazepines. Patient reports last year feeling ambivalent about sobriety and was not really committed to recovery. We have been told that his primary care physician has been notified about his addiction and has agreed not to continue prescribing Xanax. However patient reports that he was able to manipulate his primary physician back to get Xanax prescription and has been taking it consistently since last year. Per PDMP there has been consistent prescription of Xanax 1 mg twice daily throughout whole last year as well as this year.

Initial plan : discussed with patient treatment plan as well as diagnostic impression. Reviewed current medication regimen. Patient reports last use of Xanax was in early June while last use of Ambien was 3 to 4 days ago. He denies severe withdrawal symptoms from Xanax but still experiencing a lot of anxiety, insomnia, some mild tremors. He is agreeable to discontinue Ambien completely and last usage was 4 days ago. He is still experiencing a lot of insomnia and we discussed resuming trazodone which was helpful in the past. We will be resuming trazodone at 100 mg at bedtime. Otherwise continue low-dose of Focalin for ADHD while carefully monitoring for PDMP.
Patient denies mismanaging prescribed Focalin and he is at very low dosage. Reviewed his PDMP in detail. He is agreeable to sign release information form for his primary care physician. He reports that he already has notified his primary care about not wanting to get any further prescriptions of Xanax and Ambien given his addiction. We will be confirming that. He also call his pharmacy to discontinue any active prescriptions for

**Medications**
Inpatient
No active inpatient medications
Home
busPIRone 15 mg oral tablet, 15 mg, 1 tabs, Oral, BID
escitalopram 20 mg oral tablet, 30 mg, 1.5 tabs, Oral, Daily
Focalin XR 20 mg oral capsule, extended release, 20 mg, 1 caps, Oral, qAM
traZODone 150 mg oral tablet, 150 mg, 1 tabs, Oral, QHS

**Vitals & Measurements**
No Vitals Found Within 72 Hours

**Lab Results**
Urine PH: 6.2 (07/22/21 15:07:00)
Alcohol Metabolites: NEGATIVE (07/22/21 15:07:00)
Buprenorphine Urine: NEGATIVE (07/22/21 15:07:00)
6 Acetylmorphine: NEGATIVE (07/22/21 15:07:00)
MDMA: NEGATIVE (07/22/21 15:07:00)
Alphahydroxyalprazolam: 1560 ng/mL High (07/22/21 15:07:00)
Alphahydroxymidazolam: NEGATIVE (07/22/21 15:07:00)
Alphahydroxytriazolam: NEGATIVE (07/22/21 15:07:00)
Aminoclonazepam: NEGATIVE (07/22/21 15:07:00)
Amphetamine Urine: NEGATIVE (07/22/21 15:07:00)
Benzodiazepines Urine: POSITIVE Abnormal (07/22/21 15:07:00)
Cocaine Metabolite: NEGATIVE (07/22/21 15:07:00)
Creatinine Urine: 254.9 mg/dL (07/22/21 15:07:00)
Hydroxyethylflurazepam: NEGATIVE (07/22/21 15:07:00)
Lorazepam Urine: NEGATIVE (07/22/21 15:07:00)
Marijuana Metabolite: NEGATIVE (07/22/21 15:07:00)
Methamphetamine Urine: NEGATIVE (07/13/21 15:12:00)
Nordiazepam: NEGATIVE (07/22/21 15:07:00)
Opiates: NEGATIVE (07/22/21 15:07:00)
Oxazepam Urine: NEGATIVE (07/22/21 15:07:00)
Oxidant: NEGATIVE (07/22/21 15:07:00)

## *Discharge Planning Documents*

Xanax or Ambien. I will be putting a flag on his PDMP so that future providers can be notified about need to avoid any benzodiazepines or Z drugs. Given underlying anxiety and depression. Discussed optimizing and increasing Lexapro up to 30 mg daily. Reviewed risk, benefits as well as potential side effects. Urine drug screen will be monitored. Patient was encouraged to start attending 12-step meetings which he has been most recently. Family session will be held. We also discussed sober living as another option if he is failing in IOP.

Patient initially was quite engaging in our programming and has maintained sobriety. He was regularly attending 12-step meetings. His Lexapro has been adjusted to 30 mg daily with good benefit. Also trazodone was started for insomnia. I was able to notify his primary care physician about patient abusing benzodiazepines so that he can be flagged. I have also placed a flag on PDMP.

Toward the end of his programming patient did relapse on Xanax that he found in his belongings. There were also several absences toward his discharge which was definitely concerning.

Date of service: 7/29/2021 14:07:11
Patient has been seen and interviewed. Case has been discussed during treatment team staffing meeting. Group notes have been reviewed in Cerner. This appointment was conducted via telemedicine with patient at clinic and doctor's location of a secure, private, home office. Patient has been seen for follow-up appointment. He has been absent yesterday as apparently he was sleeping throughout the day. In the context of recent relapse of Xanax, we definitely had concerns. Patient however adamantly denies relapsing following his last relapse. Reports that has been spending time with girlfriend. He does have aftercare appointments including appointment wit therapist and primary care physician. Has been continuing to attend meetings.h reports ongoing anxiety about court stressors. He is still probing me to increase his Focalin which I have declined. Patient reports that his plan will be to have family members when he will be moving out of his apartment in case he finds any drugs. He does have a sponsor in community. Some anticipatory anxiety noted but he overall feels that anxiety has been improving. Reports tolerating medications without side effects. Denies suicidal ideations. Reviewed his most recent urine drug screen which is positive for Xanax following his last relapse.

**Discharge Condition /Prognosis**
Appearance Dress: Well dressed and groomed, Meticulous
Psychomotor Behavior: Gait WNL, Station WNL
Speech: Normal rate, rhythm, and volume, Spontaneous, fluent, coherent, Articulate, Coherent
Interaction: Engages easily in conversation, Establishes rapport, Eye contact consistent
Behavior: Appropriate, Calm, Cooperative, Pleasant
Affect: Congruent with mood, reactive, anxious
Mood: "Down end anxious"
Hallucinations: None
Delusions: None
Thought Content: No suicidal ideation, No homicidal ideation, No self injurious thoughts
Thought Process: Logical, Goal oriented, Linear
Memory: Able to track conversation and answer questions appropriately, Long term memory intact as evidenced by the ability to provide history consistent with past events
Level of consciousness: Alert

Oxycodone Urine: NEGATIVE (07/22/21 15:07:00)
Temazepam Urine: NEGATIVE (07/22/21 15:07:00)
Amphetamines Comments.: SEE COMMENT (07/13/21 15:12:00)
Benzodiazepines Comments.: SEE COMMENT (07/22/21 15:07:00)
Notes and Comments.: SEE COMMENT (07/22/21 15:07:00)

**Scales and Assessments**
CIWA-Ar Total Score: 7 (06/22/21 16:14:00)
Outcome Measures
QIDS-SR Total Score: 7 (07/23/21 13:34:55)
QIDS-SR Total Score: 11 (07/16/21 12:40:04)
AWARE Total Score: 93 (07/23/21 13:42:28)
AWARE Total Score: 88 (07/16/21 12:49:24)
SFQLESQ Total Score: 46.43 (06/25/21 12:38:24)
LSAS - Fear subscale: 38 (07/23/21 13:39:20)
LSAS - Fear subscale: 36 (07/16/21 12:45:43)
LSAS - Avoidance subscale: 36 (07/23/21 13:39:20)
LSAS - Avoidance subscale: 33 (07/16/21 12:45:43)
LSAS Total: 74 (07/23/21 13:39:20)
LSAS Total: 69 (07/16/21 12:45:43)
BADS - Activation subscale: 11 (07/23/21 13:41:12)
BADS - Activation subscale: 10 (07/16/21 12:47:34)
BADS - Avoidance subscale: 12 (07/23/21 13:41:12)
BADS - Avoidance subscale: 9 (07/16/21 12:47:34)
BADS Total: 17 (07/23/21 13:41:12)
BADS Total: 19 (07/16/21 12:47:34)
PSWQ Total: 54 (07/23/21 13:37:26)
PSWQ Total: 53 (07/16/21 12:44:02)
REISS_ASI Total: 13 (07/23/21 13:40:05)
REISS_ASI Total: 13 (07/16/21 12:46:33)
WSASF Total: 15 (06/25/21 12:37:32)

## *Discharge Planning Documents*

Orientation: Oriented to person, Oriented to place, Oriented to situation, Oriented to time
Concentration: Able to focus
Intelligence: Average, As evidenced by ability to reason, problem solve and plan
Insight: Fair, As evidenced by how behavior affects others
Judgement: Fair, As evidenced by recent behavior
Language: Appropriately placed inflections, Clear

**Suicide Risk Assessment**
Consolidated Suicide Risk Level: Low

Rationale for Consolidated Suicide Risk

After reviewing the results of the CSSR-S, SAFE-T, environmental risks, and clinical factors with the therapist, I assess the patient to be at low risk and will provide the team with the following orders: Strategies to mitigate the patient's risk include safety planning, involvement of support people, 1 time/day safety check-ins.

**Suicide Risk & Protective Factors**
Suicidal Ideation, Plan and Intent
CSSRS Screen Idea w-Intent No Plan: Past month, no
CSSRS Screen Suicide Intent w-Plan: Past month, no
CSSRS Suicidal Behavior Lifetime: No
CSSRS Screen Wish to be Dead: Past month, no
CSSRS Screen Suicidal Thoughts: Past month, no
CSSRS Screen Idea w-Method No Intent: Past month, no
CSSRS Recent If Yes What Method: none
CSSRS Access Lethal Means in HomeOther: No
Protective Factors
CSSRS Internal Protective Factors: Ability to cope with stress, Frustration tolerance, Fear of death or the actual act of killing self, Identifies reasons for living
CSSRS External Protective Factors: Cultural, spiritual, and/or moral attitudes against suicide, Responsibility to children, Supportive social network of family or friends
CSSRS Deterrant Rating: 5 This deterrent definitely stopped me from attempting suicide
Patient Specific Risk Factors
Non-Modifiable Risk Precipitants: Chronic physical pain or other acute medical problem, Legal problems, Inadequate social supports, Deterioration in functioning due to psychiatric illness, Unemployment, Recent loss(es) or other significant negative events
Non-Modifiable Risk Exposure: Family history of suicidal behavior, Family history of psychiatric diagnoses requiring hospitalization
Modifiable RIsk Factors Other: Perceived burdensomeness, History of non-adherence to treatment
Non-Modifiable RIsk Characteristics: Male, Widowed, divorced, or single, Caucasian
Non-Modifiable RIsk Prior Bx: N/A
Non-Modifiable Risk Change in Treatment: Recent inpatient discharge (past 3 months)
Environmental Risk Assessment / Additional Factors
CSSRS Environmental Current Living: Lives with family
Potential Home Risk Factors: Ligature, Electrical, Sharps, Liquids, Vehicles/exhaust
Environmental Risk Factors: Other: none

**Assessment/Formulation/Plan**
Patient has successfully completed MHAR IOP program. Patient was invested in recovery and has attended community meetings. Aftercare to be set up by SW. Patient felt comfortable with discharge and denied any cravings for drugs or alcohol.

## *Discharge Planning Documents*

**SAFETY ASSESSMENT OF DANGER TO SELF AND OTHERS:** Low Risk for self-harm or
suicidality. Protective factors of hopefulness, values others/relationships, engagement in treatment, spiritual beliefs.
Protective Factors (check all that apply):

[x ]absence of suicidal/homicidal ideation, intent, plan, or behaviors
[ ] absence of substance use problems
[x ] strong support system
[x ] responsibility for family, children, or pets
[x ] future oriented
[x] help-seeking.
[x ] positive therapeutic relationship
[x ] medication compliant

**Diagnoses**

1. **Moderate recurrent MDD**(Confirmed)
2. **GAD - Generalized anxiety disorder**(Confirmed)
3. **ADHD**(Complaint of)
4. **Sedative, hypnotic, or anxiolytic use disorder, Severe**(Confirmed)

**Discharge Disposition**

**Patient will have follow-up appointment with therapist as well as primary care physician. His primary care physician is aware of patient abusing prescribed benzodiazepines. In additionhe has been flagged in PDMP.**

**Status on Discharge**

Psychiatric_ Patient was psychiatrically stable on a day of discharge and was perceived to be at low risk of harm to self or others. Patient has denied suicidal or homicidal thoughts.

Medical_ No acute medical concerns. Patient is stable for discharge. Advised to follow up with PCP regarding ongoing medical care.
Functional_ Independent with no limitations in activities in daily living. Able to go back to work responsibilities.

Electronically Signed on 08/01/2021 02:10 PM CDT

Barisic-Dujmovic MD, Tatjana

Modified by: Barisic-Dujmovic MD, Tatjana on 08/01/2021 02:10 PM CDT

## Patient Information

**Patient Name:** ROSEBROOK, KYLE A
**Home Address:** 8805 W Maple St
West Allis, WI 53214
**Home Phone:** (262) 349-5504
**Employer Name:** Unknown
**Employer Phone:**

**Sex:** Male
**DOB:** 01/16/1986
**Age:** 38 Years
**Religion:** Unknown
**SSN:** XXX-XX-2138

## Guarantor Information

**Guarantor Name:** PHYLLIS ROSEBROOK
**Patient's Reltn:** Child
**Billing Address:** 17556 W Windolw Drive
Grayslake, IL 60030
**Billing Phone:** (847) 989-3275
**Employer Name:** Unknown
**Employer Phone:**

**Sex:** Female
**DOB:** 05/17/1960
**Age:** 63 Years
**SSN:**

## Contact Information

***Emergency Contact***
**Contact Name:** PHYLLIS ROSEBROOK
**Patient's Reltn:** Child - EC
**Sex:** Female
**Home Phone:** (847) 989-3275

***Next of Kin***
**Contact Name:**
**Patient's Reltn:**
**Sex:**
**Home Phone:**

## Primary Insurance

**Subscriber Name:** ROSEBROOK, KYLE A
**Patient's Reltn:** SELF
**Sex:** Male
**DOB:** 01/16/1986
**Age:** 38 Years
**Employer Name:** Unknown
**Employer Phone:**
**Financial Class:** Medicaid MCO

**Insurance Name:** MEDICAID HMO CCHP WI
**Claim Address:** PO BOX 56099
MADISON, WI 537059399
**Insurance Phone:** (800) 482-8010
**Policy Number:** 6448859365
**Group Number:**
**Authorization Number:** EPS-00487420
**Authorization Phone:**
**Authorization Contact:**

## Secondary Insurance

**Subscriber Name:**
**Patient's Reltn:**
**Sex:**
**DOB:**
**Age:**
**Employer Name:**
**Employer Phone:**
**Financial Class:**

**Insurance Name:**
**Claim Address:**

**Insurance Phone:**
**Policy Number:**
**Group Number:**
**Authorization Number:**
**Authorization Phone:**
**Authorization Contact:**

## Encounter Information

**Reg Dt/Tm:** 09/14/2020 11:22
**Est Dt of Arrival:** 09/14/2020 09:30
**Inpt Adm Dt/Tm:**
**Disch Dt/Tm:** 10/16/2020 12:00
**Observation Dt/Tm:**
**VIP Indicator:**
**Admit Reason:** Depression

**Patient Type:** Intensive Outpatient
**Medical Service:** Tele MH & Addiction
**Location:** 2-IOP MH and Addiction R
**Room/Bed:** /
**Isolation:**
**Disease Alert:**

**Admit Type:** Urgent
**Admit Source:** Clinic or Physician O
**Advance Directive:** No, Info Provide
**Reg Clerk:** Shannon Schlimgen
**Admit Physician:**
**Attend Physician:**
**PCP:**

# ROSEBROOK, KYLE A
## Male / 38 Years

**MRN:** 00153481

**FIN:** 000216549

| | | | |
|---|---|---|---|
| Patient: | **ROSEBROOK, KYLE A** | | |
| MRN: | 00153481 | Admit: | 9/14/2020 |
| FIN: | 000216549 | Disch: | 10/16/2020 |
| DOB/Age/Sex: | 1/16/1986 34 years Male | Admitting: | zzBarisic-Dujmovic MD,Tatjana |
| Location: | 2-IOP MH and Addiction Recovery Adult | Attending: | zzBarisic-Dujmovic MD,Tatjana |

## *Clinical Summary*

| | |
|---|---|
| DOCUMENT NAME: | Clinical Summary |
| SERVICE DATE/TIME: | 10/16/2020 17:06 CDT |
| RESULT STATUS: | Modified |
| PERFORM INFORMATION: | Yelvington LPC SAC,Donna (10/16/2020 17:06 CDT) |
| SIGN INFORMATION: | Barisic-Dujmovic MD,Tatjana (10/20/2020 16:39 CDT); Yelvington LPC SAC,Donna (10/16/2020 17:06 CDT); Moon CSAC APSW,Andy (10/15/2020 12:09 CDT); Moon CSAC APSW,Andy (10/14/2020 14:23 CDT); Moon CSAC APSW, Andy (10/14/2020 14:22 CDT) |

**Clinical Summary**

**Rogers Behavioral Health**

**11101 W. Lincoln Ave, West Allis, WI 53227**

**414-327-3000**

**Care Transfer Instructions**

**PERSON INFORMATION**

**Name:** ROSEBROOK, KYLE A

MRN: 00153481 **FIN#:** 000216549

**PHYSICIANS**

**Admitting Physician:** Barisic-Dujmovic MD, Tatjana

**Attending Physician:** Barisic-Dujmovic MD, Tatjana

**Discharge Diagnosis:** 1:Moderate recurrent MDD; 2:GAD (generalized anxiety disorder); 3:Cannabis use disorder, Moderate.; 4:Opioid use disorder, Severe; 5:Sedative, hypnotic, or anxiolytic use disorder, Severe

**Comment:**

**PATIENT EDUCATION INFORMATION**

| | | | |
|---|---|---|---|
| Patient Name: | ROSEBROOK, KYLE A | | |
| MRN: | 00153481 | Admit: | 9/14/2020 |
| FIN: | 000216549 | Disch: | 10/16/2020 |
| DOB/Age/Sex: | 1/16/1986 34 years Male | | |

## *Clinical Summary*

**Instructions:**

**Medication Leaflets:**

**Follow-up:**

| With: | Address: | When: |
|---|---|---|
| Erin Ehreins, Therapist | Shorewood, WI | 11/15/2020 11:00 AM |

**Comments:**
Appointment Scheduled

| With: | Address: | When: |
|---|---|---|
| Bobbi Jo Durst, Psychiatrist | | 11/14/2020 1:00 PM |

**Comments:**
Appointment Scheduled

**MEDICATION LIST**

**Medication Reconciliation at Discharge:**

**New Medications**

**PICK N SAVE PHARMACY #407, 2201 Miller Parkway West Milwaukee, WI 53214, (414) 384 - 2474**

**naltrexone (naltrexone 50 mg oral tablet)** 1 Tabs Oral (given by mouth) every day for 30 Days. Refills: 0.
Last Dose:_____

**Medications That Have Not Changed**

**PICK N SAVE PHARMACY #407, 2201 Miller Parkway West Milwaukee, WI 53214, (414) 384 - 2474**

## *Clinical Summary*

**traZODone (traZODone 100 mg oral tablet)** 1 Tabs Oral (given by mouth)
Once a Day (at bedtime) as needed insomnia. Refills: 0.
Last Dose:_____

**Other Medications**

**ARIPiprazole (ARIPiprazole 5 mg oral tablet)**
TAKE ONE TABLET BY MOUTH EVERY NIGHT AT BEDTIME. Refills: 0.
Last Dose:_____

**atomoxetine (atomoxetine 40 mg oral capsule)**
TAKE ONE CAPSULE BY MOUTH EVERY MORNING. Refills: 0.
Last Dose:_____

**Additional Medicafions Reviewed During Your Visit**

**ARIPiprazole (ARIPiprazole 5 mg oral tablet)** 1 Tabs Oral (given by mouth)
Once a Day (at bedtime). Refills: 0.

**atomoxetine (Strattera 40 mg oral capsule)** 1 Capsules Oral (given by mouth)
once a day (in the morning). Refills: 0.

**busPIRone (busPIRone 30 mg oral tablet)** 1 Tabs Oral (given by mouth)
2 times a day. Refills: 0.

**escitalopram (escitalopram 20 mg oral tablet)** 1 Tabs Oral (given by mouth)
every day. Refills: 0.

**escitalopram (Lexapro 5 mg oral tablet)** 1 Tabs Oral (given by mouth)
every day. daily dose is 25 mg. Refills: 0.

**multivitamin (Multi Vitamins oral capsule)** 1 Capsules Oral (given by mouth)
every day.

**omega-3 polyunsaturated fatty acids (Fish Oil oral capsule)** 1 Capsules Oral (given by mouth)
every day.

**Patient's Final Home Medication List Upon Discharge:**

**ARIPiprazole (ARIPiprazole 5 mg oral tablet)**
TAKE ONE TABLET BY MOUTH EVERY NIGHT AT BEDTIME. Refills: 0.

**ARIPiprazole (ARIPiprazole 5 mg oral tablet)** 1 Tabs Oral (given by mouth)
Once a Day (at bedtime). Refills: 0.

## *Clinical Summary*

**atomoxetine (atomoxetine 40 mg oral capsule)**
TAKE ONE CAPSULE BY MOUTH EVERY MORNING. Refills: 0.

**atomoxetine (Strattera 40 mg oral capsule)** 1 Capsules Oral (given by mouth)
once a day (in the morning). Refills: 0.

**busPIRone (busPIRone 30 mg oral tablet)** 1 Tabs Oral (given by mouth)
2 times a day. Refills: 0.

**escitalopram (escitalopram 20 mg oral tablet)** 1 Tabs Oral (given by mouth)
every day. Refills: 0.

**escitalopram (Lexapro 5 mg oral tablet)** 1 Tabs Oral (given by mouth)
every day. daily dose is 25 mg. Refills: 0.

**multivitamin (Multi Vitamins oral capsule)** 1 Capsules Oral (given by mouth)
every day.

**naltrexone (naltrexone 50 mg oral tablet)** 1 Tabs Oral (given by mouth)
every day for 30 Days. Refills: 0.

**omega-3 polyunsaturated fatty acids (Fish Oil oral capsule)** 1 Capsules Oral (given by mouth)
every day.

**traZODone (traZODone 100 mg oral tablet)** 1 Tabs Oral (given by mouth)
Once a Day (at bedtime) as needed insomnia. Refills: 0.

**Comment:**

**ORDERS**

| Order Name | Order Details |
|---|---|

**Discharged To and Disposition:** Outpatient therapy
Home

**Legal Information or Stipulations**

**Medical Problems during Hospital Stay, Including Labs:** None arose.

**Medical Follow Up Needed:** Follow up with your outside providers as needed and/or scheduled.

Patient Name: ROSEBROOK, KYLE A
MRN: 00153481 Admit: 9/14/2020
FIN: 000216549 Disch: 10/16/2020
DOB/Age/Sex: 1/16/1986 34 years Male

## *Clinical Summary*

**Aftercare Recommendations as made by your treatment team:** Outpatient therapy

**Aftercare Education Recommendations made by your treatment team:** Does not attend school

**Summary of Services Provided:** Interdisciplinary assessments, Experiential Therapy, Social services group, Aftercare planning, Medical stabilization, Safety plan development, Other: Nursing groups, Spiritual care groups.

**Course of Treatment:** While enrolled in this IOP group treatment episode, Kyle participated in the following: process groups, psychoeducation groups, random UDS, experiential therapy groups, spiritual care groups, nursing groups, aftercare planning.

**Goals Achieved:** Kyle reported achieving the following IOP group treatment episode goals:

" The goals i(sic) feel I've achieved while in programming is learned a multitude of coping skills and new abilities to understand my disease and come to terms with it ".

**Risk Factors:** Loss of interest in activities, Hopelessness or despair, Anxiety and/or panic, Substance abuse/dependence

Male, Widowed, divorced, or single, Caucasian

Legal problems, Deterioration in functioning due to psychiatric illness
Mood disorder, Alcohol/substance use disorder, ADHD, Traumatic brain injury, Presence of multiple psychiatric disorders
Family history of suicidal behavior, Family history of psychiatric diagnoses requiring hospitalization
Recent inpatient discharge (past 3 months)

**Protective Factors:** Ability to cope with stress, Frustration tolerance, Fear of death or the actual act of killing self, Identifies reasons for living
Cultural, spiritual, and/or moral attitudes against suicide, Responsibility to children, Supportive social network of family or friends
5 This deterrent definitely stopped me from attempting suicide

**Suicide Risk Determination:** Low

**Clinical Summary of Suicide Risk Determination:** Client is scheduled to successfully discharge from this IOP group treatment episode on 10/16/2020. This writer completed the client's suicide risk assessment for IOP discharge on 10/15/2020 since this writer will be absent from IOP group on 10/16/2020. Client identified the client's family as a reason for living. Client expressed interest in the future. Client denied history of suicide attempts. Client denied history of suicidal ideation. Client denied present thoughts, urges, plans, and actions for suicide, homicide, and self-harm. Client reported the client's brother has a history of suicide attempt.

## Patient Information

**Patient Name:** ROSEBROOK, KYLE A
**Home Address:** 8805 W Maple St
West Allis, WI 53214
**Home Phone:** (262) 349-5504
**Employer Name:** Unknown
**Employer Phone:**

**Sex:** Male
**DOB:** 01/16/1986
**Age:** 38 Years
**Religion:** Unknown
**SSN:** XXX-XX-2138

## Guarantor Information

**Guarantor Name:** PHYLLIS ROSEBROOK
**Patient's Reltn:** Child
**Billing Address:** 17556 W Windolw Drive
Grayslake, IL 60030
**Billing Phone:** (847) 989-3275
**Employer Name:** Unknown
**Employer Phone:**

**Sex:** Female
**DOB:** 05/17/1960
**Age:** 63 Years
**SSN:**

## Contact Information

***Emergency Contact***
**Contact Name:** PHYLLIS ROSEBROOK
**Patient's Reltn:** Child - EC
**Sex:** Female
**Home Phone:** (847) 989-3275

***Next of Kin***
**Contact Name:**
**Patient's Reltn:**
**Sex:**
**Home Phone:**

## Primary Insurance

**Subscriber Name:** ROSEBROOK, KYLE A
**Patient's Reltn:** SELF
**Sex:** Male
**DOB:** 01/16/1986
**Age:** 38 Years
**Employer Name:** Unknown
**Employer Phone:**
**Financial Class:** Medicaid MCO

**Insurance Name:** Self Pay
**Claim Address:**

**Insurance Phone:**
**Policy Number:**
**Group Number:**
**Authorization Number:**
**Authorization Phone:**
**Authorization Contact:**

## Secondary Insurance

**Subscriber Name:**
**Patient's Reltn:**
**Sex:**
**DOB:**
**Age:**
**Employer Name:**
**Employer Phone:**
**Financial Class:**

**Insurance Name:**
**Claim Address:**

**Insurance Phone:**
**Policy Number:**
**Group Number:**
**Authorization Number:**
**Authorization Phone:**
**Authorization Contact:**

## Encounter Information

**Reg Dt/Tm:** 06/22/2020 10:36
**Est Dt of Arrival:** 06/22/2020 11:00
**Inpt Adm Dt/Tm:** 06/22/2020 10:36
**Disch Dt/Tm:** 07/30/2020 16:05
**Observation Dt/Tm:**
**VIP Indicator:**
**Admit Reason:** Addiction

**Patient Type:** Residential
**Medical Service:** Addiction Recovery
**Location:** 1-RES MH and Addiction R
**Room/Bed:** 211 / A
**Isolation:**
**Disease Alert:**

**Admit Type:** Urgent
**Admit Source:** Home- Non-Health
**Advance Directive:** No, Info Provide
**Reg Clerk:** Sarah zzFogl
**Admit Physician:** MD Jerry Halvers
**Attend Physician:** MD Jerry Halvers
**PCP:**

# ROSEBROOK, KYLE A
## Male / 38 Years

**MRN:** 00153481

**FIN:** 000203596

| | | | |
|---|---|---|---|
| Patient: | **ROSEBROOK, KYLE A** | | |
| MRN: | 00153481 | Admit: | 6/22/2020 |
| FIN: | 000203596 | Disch: | 7/30/2020 |
| DOB/Age/Sex: | 1/16/1986 34 years Male | Admitting: | Halverson MD,Jerry L |
| Location: | 1-RES MH and Addiction Recovery Adult; 211; A | Attending: | Halverson MD,Jerry L |

## *Discharge Planning Documents*

| | |
|---|---|
| Document Type: | Discharge Summary |
| Service Date/Time: | 7/30/2020 13:56 CDT |
| Result Status: | Auth (Verified) |
| Perform Information: | Halverson MD,Jerry L (7/30/2020 15:29 CDT) |
| Sign Information: | Halverson MD,Jerry L (7/30/2020 15:29 CDT) |

**Chief Complaint**
I overdosed on Ambien and benzodiazepine and I have been addicted to benzodiazepine for some time

**Admission Information**
Kyle is a 34-year-old male that comes to us for definitive treatment of benzodiazepine and opioid dependence. There is also mood and anxiety. Overall it is clear that benzodiazepines were Kyle's drug of choice although he really does present with a polysubstance dependence picture. He has been treated for depression and anxiety in the past and he comes to us with trouble focusing. Please see my admission note for complete detail

**Course of Treatment**
Kyle did well in our program. Early on in the program Kyle had concerns with dizziness and lack of concentration. We did add Strattera which gave him some benefit and ultimately advanced the dose to 80 mg which was beneficial to him. Further he complained of depression and anxiety. We did optimize Lexapro and buspirone and his mood did stabilize. We did obviously work with him on addiction also. We did discuss the possibility of medication assisted treatment but benzodiazepines being his primary drug of choice it did not ultimately make sense. There was no clear need to treat the opiates as he had done a very good job with that. We worked on support family support and friend support as well as recovery skills. He had meetings at individual and group level to work on his addiction. We did work him through the first steps of his program. Over the course of the hospitalization his mood and anxiety improved he no longer met any sort of criteria for issues with mood and anxiety. His cravings are also improved over his hospitalization. He was to follow-up with the virtual partial hospitalization program in Kenosha.

**Discharge Condition /Prognosis**
I did see him on the day of discharge. He was hopeful he was ready for discharge. He denied suicidal or homicidal ideation and he denied active cravings

**Assessment/Formulation/Plan**
Polysubstance dependence. He will follow-up at the virtual partial hospital program in Kenosha. He has plans to attend meetings on top of that. There was no medication assisted treatment. Depression and anxiety as well as attention. We did make medication changes with the buspirone and the Lexapro as well as Strattera and he was able to discharge feeling much better. We did also continue the Abilify. This will be managed at the partial hospital level of care.

**Medications**

Inpatient
- ARIPiprazole 5 mg oral tablet, 5 mg, 1 tabs, Oral, QHS
- busPIRone, 30 mg, 1 tabs, Oral, BID
- escitalopram, 20 mg, 1 tabs, Oral, Daily
- hydrOXYzine pamoate 50 mg oral capsule, 50 mg, 1 caps, Oral, QID, PRN
- ibuprofen, 600 mg, 3 tabs, Oral, q8hr, PRN
- nicotine 2 mg oral transmucosal gum, 2 mg, 1 EA, Oral, q1hr, PRN
- Strattera 40 mg oral capsule, 80 mg, 1 caps, Oral, qAM
- traZODone 100 mg oral tablet, 100 mg, 1 tabs, Oral, QHS, PRN
- Ultimate Omega, 2 caps, Oral, Daily

Home
- Abilify, 5 mg, Oral, QHS
- ARIPiprazole 5 mg oral tablet, 5 mg, 1 tabs, Oral, QHS
- busPIRone, 30 mg, Oral, BID
- busPIRone 30 mg oral tablet, 30 mg, 1 tabs, Oral, TID, 3 refills
- busPIRone 30 mg oral tablet, 30 mg, 1 tabs, Oral, BID
- escitalopram 20 mg oral tablet, 20 mg, 1 tabs, Oral, Daily
- escitalopram 20 mg oral tablet, 20 mg, 1 tabs, Oral, Daily
- hydrOXYzine pamoate 50 mg oral capsule, 50 mg, 1 caps, Oral, QID, PRN
- hydrOXYzine pamoate 50 mg oral capsule, 50 mg, 1 caps, Oral, QID, PRN
- Strattera 40 mg oral capsule, 80 mg, 2 caps, Oral, qAM, 2 refills
- Strattera 80 mg oral capsule, 80 mg, 1 caps, Oral, qAM

## *Discharge Planning Documents*

**Diagnoses**
**Generalized anxiety disorder**(Confirmed)
**Major depressive disorder, recurrent**(Confirmed)
**Opioid use disorder, mild, in controlled environment**(Confirmed)
**Severe benzodiazepine use disorder**(Confirmed)

**Discharge Disposition**
DC Discharged To: RBH PHP Program (07/28/20 09:33:00)

**Status on Discharge**
Psychiatric_Denied suicidal or homicidal ideation. Cravings were under control
Medical_No significant medical problems at the time of discharge
Functional_Able to complete all activities of daily living at the time of discharge

traZODone 100 mg oral tablet, 100 mg, 1 tabs, Oral, QHS, PRN
traZODone 100 mg oral tablet, 100 mg, 1 tabs, Oral, QHS, PRN

**Vitals & Measurements**
Weight Measured: 87.6 kg (07/24/20 06:00:00)
Temperature Oral: 36.6 degC (07/30/20 06:00:00)
Systolic Blood Pressure Sitting: 116 mmHg (07/24/20 06:00:00)
Diastolic Blood Pressure Sitting: 84 mmHg (07/24/20 06:00:00)
Pulse Sitting: 89 bpm (07/24/20 06:00:00)

**Lab Results**
BUN: 16 mg/dL (06/25/20 04:11:00)
Sodium Level: 139 mmol/L (06/25/20 04:11:00)
Potassium Level: 4.6 mmol/L (06/25/20 04:11:00)
Chloride Level: 106 mmol/L (06/25/20 04:11:00)
Calcium Level: 9.7 mg/dL (06/25/20 04:11:00)
eGFR for African Americans: 90 mL/min/1.73 m2 (06/25/20 04:11:00)
eGFR for Non-African Americans: 78 mL/min/1.73 m2 (06/25/20 04:11:00)
Creatinine Level: 1.21 mg/dL (06/25/20 04:11:00)
CO2: 25 mmol/L (06/25/20 04:11:00)
Glucose Fasting: 101 mg/dL High (06/25/20 04:11:00)
Albumin Level: 4.5 g/dL (06/25/20 04:11:00)
Protein Total: 6.9 g/dL (06/25/20 04:11:00)
ALT/SPGT: 34 unit/L (06/25/20 04:11:00)
AST (SGOT): 23 unit/L (06/25/20 04:11:00)
Bilirubin Total: 0.4 mg/dL (06/25/20 04:11:00)
Alkaline Phosphatase Isoenzyme: 62 unit/L (06/25/20 04:11:00)
Globulin: 2.4 g/dL (06/25/20 04:11:00)
Hemoglobin A1c: 5.4 (06/25/20 04:11:00)
Albumin/Globulin Ratio: 1.9 (calc) (06/25/20 04:11:00)
BUN/Creatinine Ratio: NOT APPLICABLE (06/25/20 04:11:00)
LDL: 98 mg/dL (06/25/20 04:11:00)
Cholesterol: 179 mg/dL (06/25/20 04:11:00)
Total Cholesterol/HDL Ratio: 3.1 (calc) (06/25/20 04:11:00)
HDL Cholesterol: 57 mg/dL (06/25/20 04:11:00)
Triglycerides: 140 mg/dL (06/25/20 04:11:00)
Non-HDL: 122 mg/dL (06/25/20 04:11:00)
Urine PH: 7 (07/27/20 10:53:00)
Amitriptyline Comment: SEE COMMENT (07/27/20 10:53:00)
Alcohol Metabolites: NEGATIVE (07/27/20 10:53:00)
Buprenorphine Urine: NEGATIVE (07/27/20 10:53:00)
6 Acetylmorphine: NEGATIVE (07/27/20 10:53:00)
MDMA: NEGATIVE (07/27/20 10:53:00)
Amphetamine Urine: NEGATIVE (07/27/20 10:53:00)

## *Discharge Planning Documents*

Benzodiazepines Urine: NEGATIVE (07/27/20 10:53:00)
Cocaine Metabolite: NEGATIVE (07/27/20 10:53:00)
Codeine Urine: NEGATIVE (07/18/20 00:00:00)
Creatinine Urine: 21.3 mg/dL (07/27/20 10:53:00)
Hydrocodone Urine: NEGATIVE (07/18/20 00:00:00)
Hydromorphone Urine: NEGATIVE (07/18/20 00:00:00)
Marijuana Metabolite: NEGATIVE (07/27/20 10:53:00)
Morphine Urine: NEGATIVE (07/18/20 00:00:00)
Norhydrocodone: NEGATIVE (07/18/20 00:00:00)
Opiates: NEGATIVE (07/27/20 10:53:00)
Oxidant: NEGATIVE (07/27/20 10:53:00)
Oxycodone Urine: NEGATIVE (07/27/20 10:53:00)

**Scales and Assessments**

CIWA-Ar Total Score: 1 (06/22/20 10:32:00)
Outcome Measures
QIDS-SR Total Score: 7 (07/30/20 13:37:56)
QIDS-SR Total Score: 6 (07/09/20 20:35:01)
AWARE Total Score: 84 (07/09/20 20:47:49)
AWARE Total Score: 108 (06/23/20 20:37:39)
SFQLESQ Total Score: 76.79 (07/30/20 13:40:31)
SFQLESQ Total Score: 53.57 (06/23/20 20:13:37)
LSAS - Fear subscale: 26 (07/30/20 13:49:30)
LSAS - Fear subscale: 23 (07/09/20 20:41:12)
LSAS - Avoidance subscale: 22 (07/30/20 13:49:30)
LSAS - Avoidance subscale: 21 (07/09/20 20:41:12)
LSAS Total: 48 (07/30/20 13:49:30)
LSAS Total: 44 (07/09/20 20:41:12)
BADS - Activation subscale: 16 (07/09/20 20:45:43)
BADS - Activation subscale: 30 (06/23/20 20:28:23)
BADS - Avoidance subscale: 9 (07/09/20 20:45:43)
BADS - Avoidance subscale: 12 (06/23/20 20:28:23)
BADS Total: 25 (07/09/20 20:45:43)
BADS Total: 36 (06/23/20 20:28:23)
PSWQ Total: 55 (07/30/20 13:46:06)
PSWQ Total: 44 (07/09/20 20:38:26)
REISS_ASI Total: 17 (07/30/20 13:50:41)
REISS_ASI Total: 19 (07/09/20 20:43:57)
WSASF Total: 20 (07/30/20 13:38:59)
WSASF Total: 18 (06/23/20 20:11:50)

Electronically Signed on 07/30/2020 03:29 PM CDT

Halverson MD, Jerry L

| | | | |
|---|---|---|---|
| Patient Name: | ROSEBROOK, KYLE A | | |
| MRN: | 00153481 | Admit: | 6/22/2020 |
| FIN: | 000203596 | Disch: | 7/30/2020 |
| DOB/Age/Sex: | 1/16/1986 34 years Male | | |

## *Discharge Planning Documents*

## Patient Information

**Patient Name:** ROSEBROOK, KYLE A
**Home Address:** 8805 W Maple St
West Allis, WI 53214
**Home Phone:** (262) 349-5504
**Employer Name:** Unknown
**Employer Phone:**

**Sex:** Male
**DOB:** 01/16/1986
**Age:** 38 Years
**Religion:** Unknown
**SSN:** XXX-XX-2138

## Guarantor Information

**Guarantor Name:** KYLE ROSEBROOK
**Patient's Reltn:** SELF
**Billing Address:** 8805 W Maple St
West Allis, WI 53214
**Billing Phone:** (262) 349-5504
**Employer Name:** Unknown
**Employer Phone:**

**Sex:** Male
**DOB:** 01/16/1986
**Age:** 38 Years
**SSN:** XXX-XX-2138

## Contact Information

***Emergency Contact***
**Contact Name:** PHYLLIS ROSEBROOK
**Patient's Reltn:** Child - EC
**Sex:** Female
**Home Phone:** (847) 989-3275

***Next of Kin***
**Contact Name:**
**Patient's Reltn:**
**Sex:**
**Home Phone:**

## Primary Insurance

**Subscriber Name:** ROSEBROOK, KYLE A
**Patient's Reltn:** SELF
**Sex:** Male
**DOB:** 01/16/1986
**Age:** 38 Years
**Employer Name:** Unknown
**Employer Phone:**
**Financial Class:** Medicaid MCO

**Insurance Name:** MEDICAID HMO CCHP WI
**Claim Address:** PO BOX 56099
MADISON, WI 537059399
**Insurance Phone:** (800) 482-8010
**Policy Number:** 6448859365
**Group Number:**
**Authorization Number:** EPS-00472765
**Authorization Phone:** (414) 266-8681
**Authorization Contact:** Katie D.

## Secondary Insurance

**Subscriber Name:**
**Patient's Reltn:**
**Sex:**
**DOB:**
**Age:**
**Employer Name:**
**Employer Phone:**
**Financial Class:**

**Insurance Name:**
**Claim Address:**

**Insurance Phone:**
**Policy Number:**
**Group Number:**
**Authorization Number:**
**Authorization Phone:**
**Authorization Contact:**

## Encounter Information

**Reg Dt/Tm:** 06/07/2020 14:12
**Est Dt of Arrival:** 06/07/2020 13:30
**Inpt Adm Dt/Tm:** 06/07/2020 14:12
**Disch Dt/Tm:** 06/17/2020 15:40
**Observation Dt/Tm:**
**VIP Indicator:**
**Admit Reason:** Addiction

**Patient Type:** Inpatient
**Medical Service:** Addiction Recovery
**Location:** 1-IP Adult
**Room/Bed:** 1383 / A
**Isolation:**
**Disease Alert:**

**Admit Type:** Urgent
**Admit Source:** Home- Non-Health
**Advance Directive:** No, Info Provide
**Reg Clerk:** Joshua Bukosky
**Admit Physician:**
**Attend Physician:**
**PCP:**

# ROSEBROOK, KYLE A
## Male / 38 Years

**MRN:** 00153481

**FIN:** 000202495

| | | | |
|---|---|---|---|
| Patient: | **ROSEBROOK, KYLE A** | | |
| MRN: | 00153481 | Admit: | 6/7/2020 |
| FIN: | 000202495 | Disch: | 6/17/2020 |
| DOB/Age/Sex: | 1/16/1986 34 years Male | Admitting: | Salami MD,Oludamilola |
| Location: | 1-IP Adult; 1383; A | Attending: | Salami MD,Oludamilola |

## *Discharge Planning Documents*

| | |
|---|---|
| Document Type: | Discharge Summary |
| Service Date/Time: | 6/17/2020 12:47 CDT |
| Result Status: | Auth (Verified) |
| Perform Information: | Salami MD,Oludamilola (6/17/2020 12:53 CDT) |
| Sign Information: | Salami MD,Oludamilola (6/19/2020 13:28 CDT) |

**Chief Complaint**
"I need to get clean and stop taking Xanax"

**Reason for Admission:**
Kyle Rosebrook is a 34-year-old male admitted for benzodiazepine withdrawal and mood instability.

**Admission History:**
Patient was found by his mother following an accidental overdose on an unspecified quantity of alprazolam and zolpidem approximately a week ago. He was found unresponsive by his mother who subsequently called EMS. Patient was hospitalized at a local hospital in Illinois for 2 days and subsequently discharged to his parents custody and has been residing with his parents prior to contacting Rogers for further management. Patient reports working memory deficits over the past week and was somewhat uncertain as to his last use of alprazolam prior to this hospitalization. Patient reports initial use of alprazolam over 15 years ago while in college. He admits that his substance use increased rapidly and he was unable to finish college due to use of Xanax. He states that longest period of abstinence has been for approximately 6 months at around 2015/16. He has since struggled to remain abstinent for any appreciable periods of time. He reports daily use of unspecified quantities of alprazolam. He also reports previous use of marijuana. Patient denies a history of substance withdrawal seizures, substance related hallucinosis or delirium tremens. He has had difficulty controlling consumption and has tried to cut down unsuccessfully on several occasions. He admits to spending significant quantities of time and effort procuring alprazolam. He has had moderate to severe withdrawal symptoms between periods of benzodiazepine use. Patient acknowledges that alcohol use has led to impairment in several domains of function including interpersonal relationships. He has recent OWI with a pending court date. He has had a history of mood dysregulation throughout his adult life. Patient admits that his mood symptoms are often worse in the context of benzodiazepine abuse. He describes feeling sad and depressed. He also reports anxiety, anhedonia, difficulty concentrating, diminished motivation and intermittent nihilistic thoughts. He denies active suicidal ideations, intent or plan. He denies homicidal thoughts. He denies history of perceptual abnormalities. He denies symptoms suggestive of mania or hypomania.

**Hospital Course:**
Patient was admitted to the Adult inpatient unit in stable medical condition. Patient was assessed as a 34-year-old male with a genetic diathesis for mood and substance use

**Medications**
Home
- Abilify, 5 mg, Oral, QHS
- busPIRone, 30 mg, Oral, BID
- escitalopram 20 mg oral tablet, 20 mg, 1 tabs, Oral, Daily
- traZODone 100 mg oral tablet, 100 mg, 1 tabs, Oral, QHS, PRN

## *Discharge Planning Documents*

disorder based on his family history. He presents with symptoms consistent with benzodiazepine use disorder and comorbid major depressive disorder. Patient completed medically supervised withdrawal from benzodiazepines using the anticonvulsant method with carbamazepine. Motivational interviewing and substance use counseling provided aimed at relapse prevention. The treatment team maintained appropriate safety precautions. Insight based therapy and supportive therapy provided aimed at maladaptive responses to psychosocial stressors. The recent and ordered laboratory tests were reviewed. The medicine consult service assisted with the management of pertinent medical co-morbidities. Resumed his most recent medications including Abilify, buspirone and escitalopram. Trazodone was added as needed at bedtime for insomnia with good therapeutic response. The indication, benefits and possible adverse effects of prescribed medications were reviewed with the patient, patient verbalized understanding and agreed to the recommended pharmacological management. Behavioral activation and other pertinent psychotherapeutic programming were provided over the course of the hospitalization aimed at improved coping skills and emotional regulation. There was a family session recommended aimed at psychoeducation, identification of support systems and discharge planning. Patient's clinical sxs improved over course of admission. No medication side effects were noted. Recommended residential dual diagnosis treatment and account of unsuccessful attempts at abstinence at lower levels of care. The hospital course was otherwise unremarkable.

**Mental status on discharge:**
Appearance Dress: Age appropriate, Well nourished
Psychomotor Behavior: No significant psychomotor changes
Speech: Normal volume, rate and latency
Behavior: Appropriate and engaged
Affect: Broader range and congruent with mood.
Mood: Improved
Perceptual abnormalities: Denies auditory, visual, tactile or gustatory misperceptions
Thought Content: No homicidal ideation. No suicidal ideation.
Thought Process: Logical, Linear
Memory: Able to track conversation and answer questions appropriately.
Level of consciousness: Alert
Orientation: Oriented to person, Oriented to place, Oriented to situation, Oriented to time
Concentration: Able to focus
Intelligence: Average, As evidenced by adequate fund of knowledge, As evidenced by ability to learn from experience
Insight: improved, As evidenced by ability to recognize own illness, As evidenced by how treatment is helpful
Judgement: improved, As evidenced by recent behavior
Strengths: Verbal, Physically healthy
Language: Clear, Spontaneous

**Status on Discharge:**
1. Psychiatric Function- Patient demonstrated improving coping mechanisms and was tolerating medication adjustments. Patient was assessed NOT to be at imminent risk of harm to self or others.
2. Medical function- acute and chronic medical comorbidities were stable and appropriately managed
3. At time of discharge, patient's functional status had improved, with the expectation that patient would be able to meet the challenges associated with getting through a regular day

| | | | |
|---|---|---|---|
| Patient Name: | ROSEBROOK, KYLE A | | |
| MRN: | 00153481 | Admit: | 6/7/2020 |
| FIN: | 000202495 | Disch: | 6/17/2020 |
| DOB/Age/Sex: | 1/16/1986 34 years Male | | |

## *Discharge Planning Documents*

**Discharge Disposition:**
**With:** Oconomowoc - Rogers Behavioral Health
**Address:** business (1), 34700 Valley Road, Oconomowoc, WI, 530664500;8007674411 Business (1)
**When:** 06/22/2020 10:00
**Comment:** Patient to admit to the Herrington Center for Addiction Recovery at Rogers Behavioral Health, Oconomowoc. Admit Date/Time: Monday June 22nd @ 10:00AM

**Risk Mitigation**
1. Patient aware to call EMS if feeling at risk of harm to self or others
2. Patient aware of available community support networks.
3.Patient advised to remove firearms, if any, from home and limit access

>30 minutes spent on counseling, documentation and coordination of care.

Patient was seen via a secure telemedicine platform, with the patient located in a private, monitored conference room on the acute adult inpatient unit at Rogers Memorial Hospital in Oconomowoc Wisconsin. The provider was located in a secure medical office, on campus, at Oconomowoc Wisconsin

**Diagnoses**
1. **Benzodiazepine use disorder**(Confirmed)
2. **MDD (major depressive disorder)**(Confirmed)

Electronically Signed on 06/19/2020 01:28 PM CDT

Salami MD, Oludamilola